



RAPID RESPONSE AND ICU TRANSFER

Document ID: SU-017-FLOW-01 | Parent Policy: SU-017 — Intensive Care Unit Admission Criteria for Stroke Patients | Version: 1.0 |
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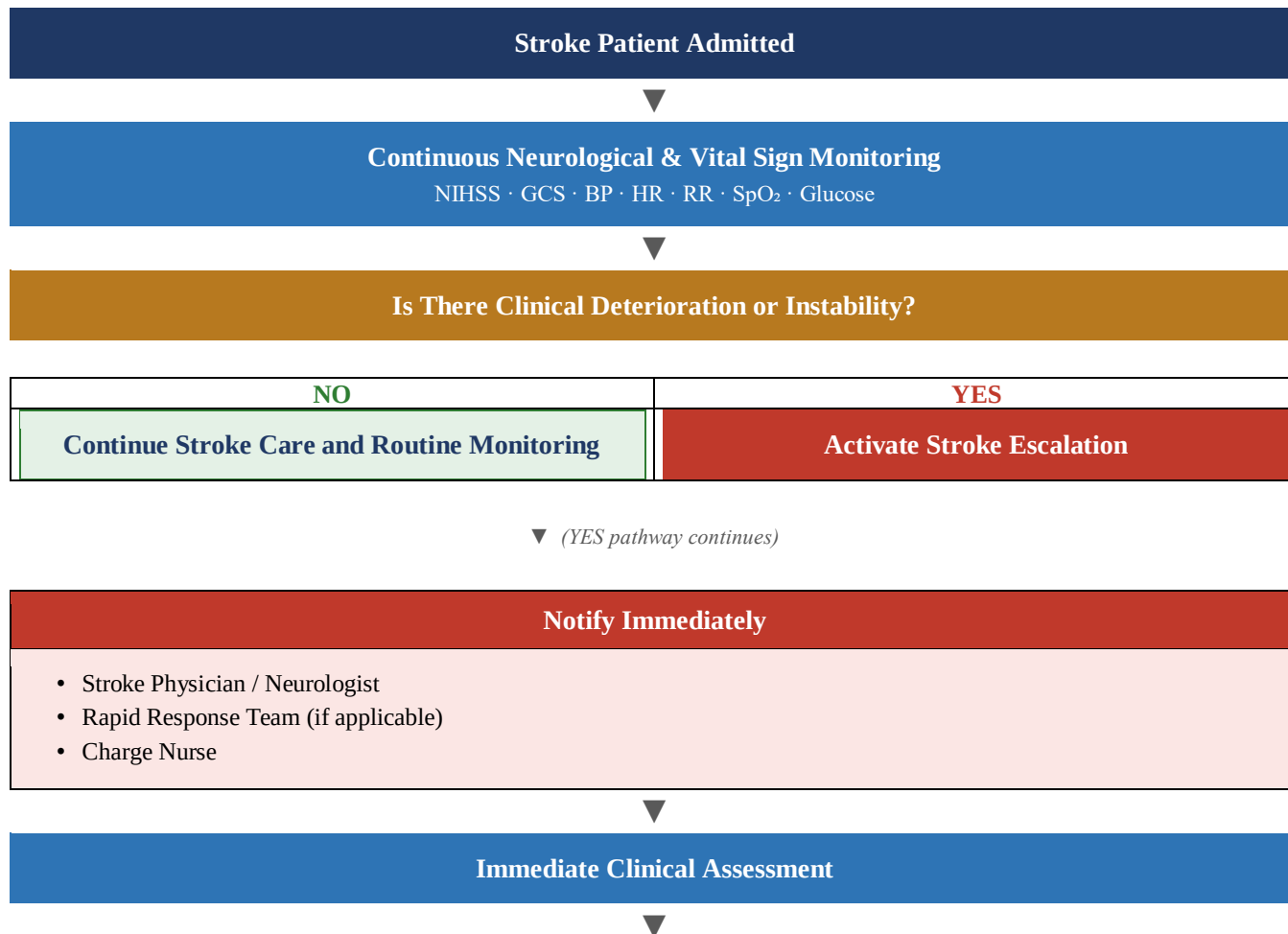
FLOWCHART FOR STROKE PATIENTS

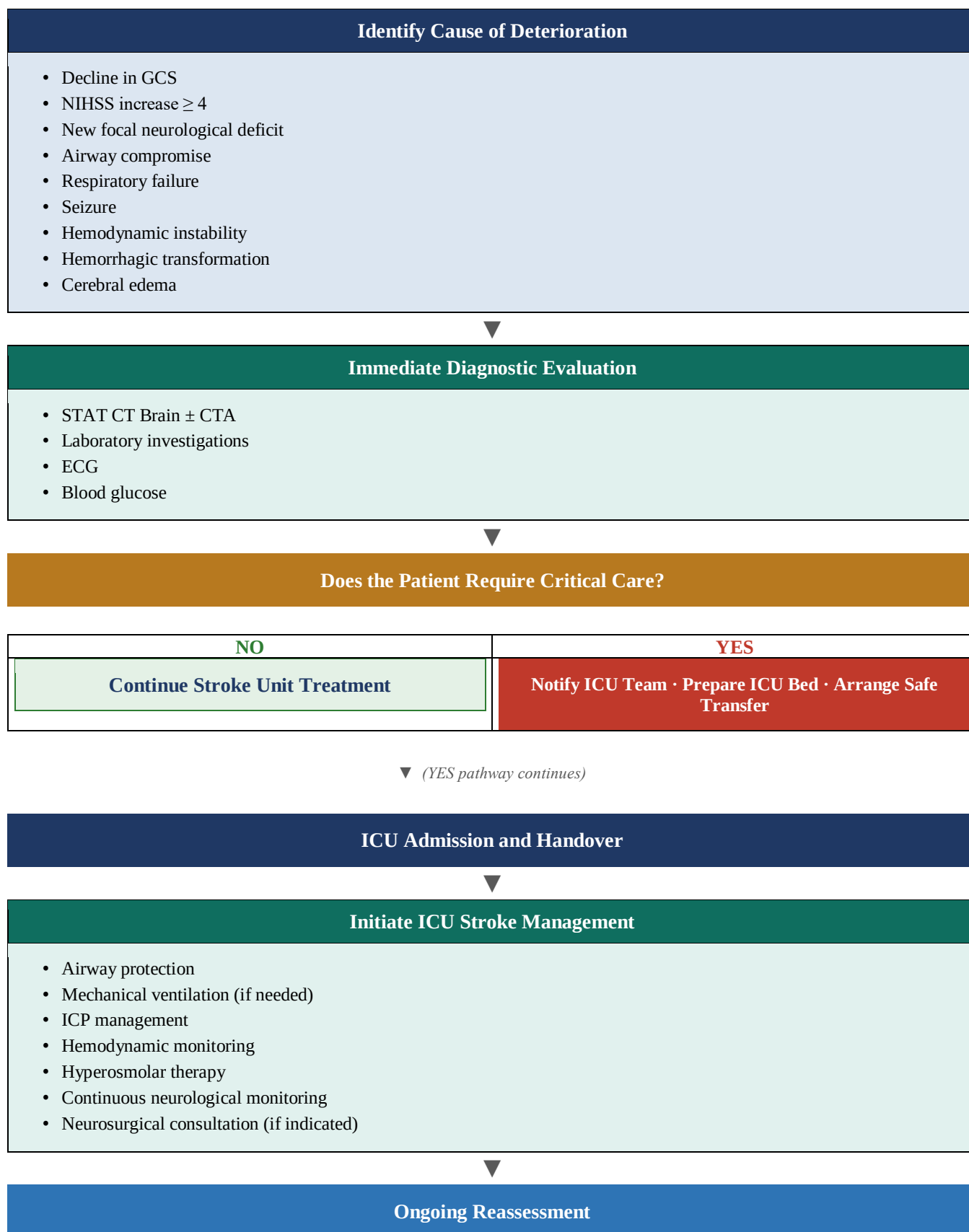
Stroke Unit — Clinical Escalation Pathway

Purpose

- To provide a standardized process for the rapid recognition, escalation, and transfer of stroke patients who develop neurological or medical deterioration requiring higher-level care.

Escalation Pathway





ICU Transfer Criteria

Transfer to the ICU should be considered for patients with one or more of the following:

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| • Decreased level of consciousness (GCS $\leq$ 12 or rapidly declining) |
| • NIHSS worsening (e.g., increase $\geq$ 4 points) |
| • Airway compromise or need for intubation |
| • Respiratory failure |
| • Hemodynamic instability requiring vasoactive support |
| • Large hemispheric infarction with cerebral edema |
| • Intracerebral hemorrhage requiring intensive monitoring |
| • Hemorrhagic transformation after thrombolysis or thrombectomy |
| • Status epilepticus or recurrent seizures |
| • Need for invasive neurological or cardiovascular monitoring |
| • Requirement for hyperosmolar therapy or intracranial pressure management |
| • Immediate neurosurgical intervention |

Clinical Note

- This flowchart is intended to support, not replace, clinical judgment. All escalation decisions should be documented in the patient record, including time of notification and response.